Sunitinib 37.5 mg · pancreatic neuroendocrine tumour (NET)

Your neuroendocrine tumour treatment Progressive unresectable or metastatic well-differentiated pancreatic neuroendocrine tumour under NCCP 00327a.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous daily sunitinib · pancreatic neuroendocrine tumour (NET)

Active medicines: Sunitinib

Emetogenic risk: Oral minimal-to-low

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Use the unit's gentle skin and mouth-care plan. Report hand-foot pain before it limits walking or daily tasks; the team may adjust treatment.
- This oral regimen is minimal-to-low emetogenic. Routine premedication is not required. Agree rescue anti-sickness and antidiarrhoeal medicines and when to call.
- Follow the agreed blood pressure (BP) plan. If you use diabetes medicines, ask how to monitor glucose and manage low readings; sunitinib may lower blood sugar.
- Report poor intake, weight loss or troublesome fatigue. Thyroid changes, anaemia, dehydration or other causes may need treatment.
- No routine premedication is required by National Cancer Control Programme (NCCP). Any antidiarrhoeal, rescue antiemetic or other support must follow the local prescription.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Sources checked 2026-11-05 · SACTCheck v0.84.37 · Regimen-level drafts; prescriber and oncology-pharmacy verification required